WRITING SAMPLE · POLICY ANALYSIS & PROGRAM EVALUATION

Utilization and Cost of Supervised Visitation

A mixed-methods analysis of statewide utilization, expenditure, equity and decision-making practice, with a comparative review of other state approaches.

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Introduction

This memorandum provides an in-depth analysis of the use of supervised visitation within the Indiana Department of Child Services (DCS). Because supervised visitation represents a substantial portion of the agency's budget, leadership sought to understand the specific applications of these services and the decision-making processes involved in determining the appropriate dosage and level of supervision required. The memorandum also offers context by examining the practices and strategies employed by other states.

Supervised parenting time is also known as supervised visitation, parent–child visitation and visitation time. Despite the different terms used, they all refer to the same concept.

Literature review

Best practices in supervised visitation

In the broader research literature on best practices for supervised visitation, there is no single set of universally accepted guidelines. However, various literature reviews are available, and it is possible to outline categories of practices associated with parent–child visitation, as well as the presumed best practices that have been recommended.

State-level cost savings initiatives

Various states have developed their own innovative approaches to supervised visitation. Outlined below are a few examples.

Washington

The Strive Supervised Visitation Program focuses on evidence-based practices and community partnerships to reduce costs. The goal is to modernize Washington State's visitation model using evidence-informed practices to support earlier permanency by increasing reunifications and decreasing the time required for a child to safely reunify with their parents. Supported by the Department of Children, Youth and Families' performance-based contracting initiative, the Family Time development is informed by recent pilot studies. Developed based on best practice literature and stakeholder input, the Strive model provides intensive and individualized services to families, focusing on early parent engagement through a welcoming environment, in-home parenting education, positive interaction modeling, helpful feedback to parents, and observing parent–child interactions for progress. This approach aims to achieve significant cost savings while ensuring effective and supportive supervised visitation (Graham, 2021).

Texas

Texas has restructured its parent–child visitation models to include community-based services and volunteer programs, which help reduce costs. The visitation environment should be as natural and home-like as possible while ensuring the child's safety, and the setting should facilitate meaningful parent–child interaction. Examples of visitation environments, ranging from most to least restrictive, include department offices, contracted provider offices, community-based centers, volunteer-supervised programs, and public places conducive to quality parent–child interaction such as parks, libraries, museums, playgrounds and quiet restaurants. Foster parents' homes (considering the safety of other foster children present and confidentiality concerns), family members' homes and parents' homes are also options. Incorporating community-based services and volunteer programs can provide a more cost-effective solution while maintaining a supportive and safe environment (Graham, 2021; Texas Department of Family and Protective Services, 2015).

Minnesota

Minnesota maximizes resources and cuts costs by implementing flexible scheduling and group visits. Flexible scheduling can lower expenses and boost effectiveness by improving compliance and removing barriers to missed visitation, such as changes in parents' work schedules or misunderstandings about the arrangements. Parents must be informed of any changes to the visitation schedule, and some visitors may be added to or removed from the visitation plan. Visitation plans are re-evaluated every six months, but may be reviewed sooner if a parent is absent from visits or if a child has a significant reaction to visits (Graham, 2021; Krenik, 2005).

California

California has designated “nonprofessional providers” as any person who is not paid for providing supervised visitation services. Unless the court says otherwise or both parties agree, the nonprofessional provider must have no convictions for child molestation, child abuse or other crimes against a person. They must have car insurance if transporting the child, and have no current or past court orders under which they were being supervised. They must agree to follow and enforce the court order regarding supervised visitation. The nonprofessional provider should be at least 21 years old and have no driving-under-the-influence convictions in the past five years. They should not have been on probation or parole for the past ten years and should have no restraining orders in the past ten years. They must not be financially dependent on the person being supervised, and must sign a local court form confirming they meet all the requirements (Graham, 2021; *Uniform standards of practice for providers of supervised visitation*, 2021).

Utah

Utah focuses on using community resources and volunteer programs to support supervised visitation, thereby reducing costs. If the court finds evidence of domestic violence, child abuse or an ongoing risk to a child and orders supervised parent-time, it shall prioritize supervision by a professional individual or private agency trained in relevant laws and needs. If such professionals or agencies are not available, affordable or practicable, the court shall prioritize supervision by an individual capable and willing to ensure the child's safety and trained in pertinent areas. If none of these options is feasible, or if there is no evidence of ongoing risk, the court may order supervised parent-time by an individual willing to protect the child, giving preference to suggestions by the parties, including relatives. At the time of imposing supervised parent-time, the court shall consider the cost of professional services and whether the supervision requirement should expire after a set period (*Custody, parent-time, and visitation*, 2024; Graham, 2021).

Factors in deciding visitation and level of need

When determining the appropriate amount of visitation and the level of supervision needed for a child and their parent, several specific factors must be considered to establish level of need. These include parent characteristics (such as history of violence, substance use or mental health issues), child characteristics (such as age, developmental needs and emotional well-being), and case characteristics (such as the presence of protective orders, history of domestic violence and past involvement with child protective services).

Supervised visitation cases typically fall into one of four categories. First, one or both parents may exhibit inappropriate behavior that prevents shared parenting. Second, there may be a risk of parental kidnapping or interference with non-custodial parent access. Third, the parent's capacity to safely care for the child may be impaired by mental health or substance abuse issues. Finally, incidents of domestic violence between the parents may necessitate supervised visitation (Babb et al., 2009; Brandt, 2007).

According to the *Faulkner Law Review*, domestic violence is a significant reason for supervised visitation referrals, with factors such as frequency, severity and physical injury influencing how children are affected. In these cases, supervised visitation is identified as a means to protect the vulnerable parent and the children, while simultaneously promoting a relationship between the parent responsible for domestic violence and their child. Researchers emphasize the importance of preventing individuals responsible for domestic violence from continuing to involve their children in abuse after separation or divorce. These individuals may also disrupt the children's relationship with their mothers, even post-separation. In cases involving domestic violence, supervised visitation programs in a facility offer advantages over supervision by a friend, family member or less qualified facilitator, who may be vulnerable to threats or intimidation (Oehme & O'Rourke, 2012).

Structured decision-making frameworks

National child welfare experts have developed structured decision-making frameworks to guide the amount of supervised visitation and the level of service provided. One prominent example is the Structured Decision Making model, developed by Evident Change. Like Indiana, many other states use the tools within this model to guide the amount of supervised visitation and the level of service provided. The model provides a suite of research-based assessment tools designed to assist child welfare staff at critical decision points throughout a case, including assessments for intake, safety, risk, family strengths and needs, and reunification (Evident Change, n.d.).

Guiding principles for supervised visitation programs

The National Council of Juvenile and Family Court Judges has developed guiding principles for supervised visitation programs through the Justice for Families Supervised Visitation Program. These principles emphasize the safety of children and adult victims of domestic violence and aim to create a supportive and secure environment for supervised visitation. The six guiding principles are safety, multiculturalism and diversity, understanding domestic violence, respect and fairness, collaboration, and advocacy.

Safety emphasizes that visitation centers must prioritize the safety of both children and adult victims equally. As a critical service during the high-risk period of parental separation, visitation centers should integrate safety into all their practices and planning. While they cannot eliminate all risks associated with domestic violence, they can significantly enhance safety through careful planning and community collaboration. Supporting adult victims helps improve the well-being of children exposed to domestic violence, creating a safer environment for everyone involved.

Multiculturalism and diversity holds that visitation centers should be responsive to the diverse backgrounds and cultures of their community and the families they serve. Recognizing and valuing these differences can improve safety and outcomes for children, adult victims and those who have used violence. This principle emphasizes the importance of understanding and incorporating cultural perspectives into the center's practices.

Understanding domestic violence emphasizes that visitation centers should have a deep understanding of the nature, dynamics and impact of domestic violence. This knowledge should be integrated into their services to better recognize and address abusive behaviors, helping ensure the safety and well-being of children and adult victims by recognizing tactics used by abusers and responding appropriately to the needs of victims.

Respect and fairness emphasizes that visitation centers should treat everyone with respect and fairness, taking into account the abuse that has occurred. The goal is to ensure the safety of children and adult victims during visits. Understanding each family's unique circumstances, such as poverty or homelessness, helps build trust. While

addressing abuse, centers should avoid dehumanizing the abuser, ensuring fair treatment to enhance safety for all involved.

Collaboration emphasizes that visitation centers should work with community partners to ensure the safety of children and adult victims while holding abusers accountable. This cooperation addresses family needs holistically and tackles issues contributing to domestic violence. By collaborating with courts, social services and other resources, visitation centers can provide effective, safe support for families.

Advocacy emphasizes that visitation centers should work with community collaborators to ensure children and adult victims have meaningful access to services, and should actively link them to those services. Advocacy involves understanding the circumstances and experiences of violence and abuse faced by children and adult victims in order to provide accurate information and referrals, including connections to trained domestic violence service providers and other supportive resources. Effective advocacy relies on supportive community conditions that prioritize victim safety and hold abusers accountable (Justice for Families Supervised Visitation and Safe Exchange Program, 2018).

Court guidelines

Courts also play a significant role in structured decision-making for supervised visitation. The National Council of Juvenile and Family Court Judges has published guidelines for courts alongside the Center for Court Innovation, aiming to reduce the risk of violence for survivors and their children. These guidelines help courts effectively coordinate with supervised visitation programs and ensure that the safety and well-being of children are prioritized.

The first guideline is **communication**. Courts and supervised visitation program staff should meet regularly to establish a close working relationship, ensuring mutual understanding of needs and concerns. Courts must provide detailed information when ordering visitation, including specific services, contact details and reporting requirements. They should also share protective orders and records of abuse with program staff. Programs must supply the court with a clear set of referral criteria and provide regular reports on visitation progress to maximize safety and efficiency.

The second guideline, **security**, stresses the importance of strong partnerships with local law enforcement agencies to keep domestic violence survivors and their children safe during visitation, especially in emergencies. Programs should have written protocols detailing expected responses from law enforcement. Ensuring a safe and secure physical space is also vital. This includes protocols for terminating visits when rules are violated, informing staff of protective orders, and managing arrival and departure times to prevent contact between parents. Facilities should have separate areas for custodial and visiting

parents, weapon screening, consistent monitoring, and maintained confidentiality for the custodial parent and child.

The third guideline, **training**, emphasizes the need for ongoing training of supervised visitation staff on the unique dangers posed by domestic violence perpetrators. Courts can conduct cross-training for agency staff and security officers on domestic violence issues. Training should equip staff to identify and respond to safety risks, such as violations of visitation terms, child distress, threats from violent parents, and volatile situations. Topics should include confidentiality, the potential for continued abuse during visits, the dynamics of domestic violence, its impact on children, legal interventions, and typical batterer behaviors. Awareness of these factors ensures objectivity in supervised visitation reports and informed decision-making in custody and visitation matters (Moore & Ford, 2006).

Since these guidelines were published, several cases have produced case law on supervised visitation. In *Morris v. Horn* (Alaska, 2009), the court ruled that a prior protective order does not conclusively prove domestic violence, so supervised visitation was improperly ordered. In *Parks v. Parks* (Alaska, 2009), supervised visitation could not automatically change to unsupervised without proof of completion of a batterer intervention program. *Saville v. Ude* (North Dakota, 2009) upheld extended protection orders requiring supervised visits due to ongoing domestic violence concerns. In *Childbert v. Soler* (New York, 2010), the court stated that a parent's inability to control anger could justify supervised visitation. The court in *Misyura v. Misyura* (Alaska, 2010) found that delegating authority to the custodial parent to arrange supervised visits or intervention programs was invalid. *Wee v. Eggener* (Alaska, 2010) affirmed unsupervised visitation despite a history of domestic violence where the parent did not pose a threat. *Wolt v. Wolt* (North Dakota, 2010) restricted the father to supervised visitation due to behavior that endangered the child, even without triggering the domestic violence presumption (Robinson, 2011).

Indiana Association of Residential Child Care Agencies and visitation decision-making

The Indiana Association of Residential Child Care Agencies bases its decisions on visitation on best practices and research. The association has developed comprehensive guidelines and resources that draw on its expertise and collaboration with organizations such as the National Council of Juvenile and Family Court Judges and the National Building Bridges Initiative.

The association emphasizes that supervised parenting time should ensure child safety and promote reunification, with the level of supervision tailored to the family's needs. Depending on the referral level, providers will monitor, coach, model and supervise visits, intervening if necessary. The higher the family's needs and challenges, the greater the supervision required. This supervision level is not static and can be adjusted through the visitation plan developed by the Child and Family Team.

Levels of supervision defined by the association

Standard supervision applies to parents free from past drug use, without untreated mental health issues, with no history of perpetrating violence, and no violations of protective orders. These parents may need guidance in nurturing parenting.

Enhanced supervision is for parents with a history of abuse, active addiction, or extended separation from the child, requiring ongoing monitoring and structured intervention.

Therapeutic supervision is for parents with significant issues such as mental illness, active addiction or severe abuse, necessitating therapeutic interventions to ensure safe interactions, including pre- and post-visit clinical sessions.

This approach ensures that supervision levels are tailored to provide the safest and most supportive environment for the child (Indiana Association of Residential Child Care Agencies representative, personal communication, December 2024; *Recommended guidelines for referrals for supervised parenting time*, n.d.).

Laws governing the provision of supervised visitation

Supervised visitation is primarily governed by state laws, though these laws are often influenced by federal regulations (Child Custody and Support Law Center, 2024). Federal laws set foundational standards and guidelines that state regulations can build upon to ensure the safety and well-being of children during supervised visitation.

One significant federal initiative is the Access and Visitation Grant, which provides federal funding for supervised visitation programs. To access this funding, state programs must adhere to the regulatory standards for program operations. These standards require states to monitor all programs funded under the grants to ensure they provide authorized services, are conducted effectively and efficiently, comply with federal evaluation and reporting requirements, and contain safeguards to ensure the safety of parents and children (Office of Child Support Services, 2021).

The **Adoption and Safe Families Act** emphasizes the safety and well-being of children, encouraging states to expedite permanency planning for children in foster care. This act can significantly influence state policies on supervised visitation, particularly in cases where the child's safety is a concern.

The **Indian Child Welfare Act** provides specific guidelines for the placement of Native American children in foster or adoptive homes. It also affects supervised visitation arrangements in cases involving Native American families, ensuring that the cultural and familial ties of Native American children are respected and maintained.

The **Child Abuse Prevention and Treatment Act** provides federal funding to states for child abuse prevention and treatment programs, and can influence state laws and policies related to supervised visitation, especially in cases with a history of abuse or neglect.

While the **Family Educational Rights and Privacy Act** primarily focuses on educational records, it can affect supervised visitation in cases where educational records are relevant to custody decisions, ensuring that the privacy of those records is maintained.

The **Violence Against Women Act** provides protections for victims of domestic violence and can influence supervised visitation arrangements where there is a history of domestic violence, ensuring that the visitation process includes necessary safety measures.

State spending on supervised visitation

State spending on supervised visitation programs differs significantly across the country. Each state determines its own budget allocations based on local needs, priorities and available resources. To help ensure that these critical services are accessible and effective, federal funding is also available, principally through the Access and Visitation Grant Program.

The Access and Visitation Grant Program, administered by the Administration for Children and Families, provides approximately $10 million in mandatory grant funding each year across the states. This federal support is vital in helping states develop and maintain supervised visitation programs. The funding helps cover the costs of program operations, training for supervisors, and other essential resources needed to provide high-quality services.

In addition to federal grants, individual states often allocate additional resources from their own budgets. This combined funding approach allows for a more robust and comprehensive provision of services, addressing the specific needs of families and children in different regions (Office of Child Support Services, 2023b).

The FY 2022 Access and Visitation Grant Program update provides an overview of the program's activities and impact. The program aims to increase noncustodial parents' access to and time with their children. In FY 2022 it provided approximately $10 million in funding to states and territories. Each state and territory determines how it will count services for the purposes of the annual report, and program data is not subject to validation or audit beyond what is completed at the state level.

Key highlights, FY 2022 Access and Visitation Grant Program

- **Annual participation:** over 66,000 parents and guardians participated, translating to support for around 70,000 children.
- **Participant demographics:** the largest participant groups were noncustodial fathers and custodial mothers. Most participants had never been married to the other parent, and nearly one-fifth reported an annual income of less than $10,000.
- **Services provided:** parent education was the most frequently provided service at 49%, followed by mediation at 14% and supervised visitation at 14%. In total the program supported 13,027 participants of supervised visitation programs in FY 2022.
- **Increased parenting time:** states reported that nearly 27,000 noncustodial parents benefited from grant services, resulting in increased parenting time. Among these parents, 74% of noncustodial fathers and 84% of noncustodial mothers experienced more parenting time.
- **Referral sources:** courts and child support agencies were the primary sources referring participants to the program (Office of Child Support Services, 2023a).

State limitations on the use of supervised visitation

Some states impose limitations on the use of supervised visitation based on specific criteria such as the presence of domestic violence, substance abuse or other risk factors. These limitations are typically designed to ensure the safety and well-being of the child while balancing the rights of the non-custodial parent.

For example, Texas requires case managers to identify expectations for parents during visits, as well as behaviors that could result in visits being ended or restricted. Expected behaviors for successful visitation include specifying physical and verbal boundaries; exchanging information such as notes, documents, gifts or toys; calling in advance to confirm the intention to miss a visit; remaining sober throughout the visit; refraining from using physical discipline; and refraining from bringing other persons to the visit without prior agency approval (Texas Department of Family and Protective Services, 2015).

If there is a history of domestic violence, supervised visitation may be mandated to take place in a secure and neutral location with trained professionals present to monitor the interactions. Similarly, if substance abuse is a concern, the parent may be required to undergo regular drug testing, and the visits might occur in a controlled environment. These measures help mitigate risks while still allowing the non-custodial parent to maintain a relationship with their child under safe conditions (Moore & Ford, 2006).

The categorizations specified in Indiana's service standard are relatively broad compared with the specific risk factors mentioned above. Indiana's criteria include all children and families with substantiated cases of abuse or neglect, children with the status of CHINS or juvenile delinquent/juvenile status offender, and their foster and kinship families.

Narrowing these categories could potentially assist in cost reduction by focusing resources on the highest-risk cases. For example, limiting supervised visitation to situations involving clear evidence of ongoing risk factors such as domestic violence or severe substance abuse might reduce the number of cases requiring supervision. However, this approach would need to balance cost savings against the necessity of ensuring the safety and well-being of children in need. Ultimately, Indiana could consider a more targeted approach that prioritizes supervised visitation for the most at-risk groups while maintaining flexibility to address other cases as needed (Indiana Department of Child Services, 2022).

Equitable supervised visitation

Equitable supervised visitation involves addressing several key concerns, including access to services, cultural sensitivity, and ensuring fair treatment for all families. Access to services is a primary concern, as funding and resources can significantly affect the availability of supervised visitation programs. Some jurisdictions in Ohio, Vermont, Illinois and Texas, among others, are working to ensure that these services are adequately funded and accessible to all families regardless of their financial situation. Many areas are also developing community-based supervised visitation programs to reduce costs and make services more accessible.

Cultural sensitivity is another important aspect. Many jurisdictions are ensuring that staff and volunteers are trained in cultural competency to better serve diverse populations. For example, the Safe and Sound Visitation Center in Cleveland, Ohio works with a local group, Journey Center, and its diversity, equity and inclusion core team, and conducts intakes before service begins as a way to gain information about families' needs (Darling & Wingenfeld, 2022). The All About Kids program in Saint Albans, Vermont, serving primarily rural areas, uses Inspire Action for Social Change training and materials; this center prioritizes accessibility by not setting fixed hours of operation, instead meeting families where they are and working around their schedules (Snyder et al., 2022). The Children's Home and Aid Family Visitation Center in Bloomington, Illinois meets the diverse needs of its families by completing an orientation with each individual participant to gather the unique characteristics of their family, and by establishing a variety of community partnerships (McCormick, 2022). The Planet Safe program at Safe Alliance in Austin, Texas emphasizes trauma-informed care in all aspects of service delivery by providing and attending training regularly, and familiarizing its workers with the families in their community (Herrera & Rodriguez, 2023).

Training of this kind involves understanding and respecting different cultural beliefs and practices. Areas are also providing language interpretation services to ensure that non-English speaking families can fully participate. In Indiana, providers must respect the cultural and religious diversity of the children and families they serve. All staff members who interact with these families must be aware of and sensitive to the children's cultural, ethnic and linguistic differences, and to the sexual and gender orientations of the children they serve. Services must adhere to the principles outlined in the Indiana LGBTQ Practice Guidebook. Staff are required to use neutral language, create a trust-based environment for disclosure, and maintain confidentiality. Efforts must be made to employ or have access to staff and volunteers representative of the community served, minimizing existing barriers. Contractors must develop and maintain cultural competence within their programs, including the recruitment, development and training of staff and volunteers, and use appropriate community resources and informal networks to support cultural connections (Indiana Department of Child Services, 2022).

Ensuring fair treatment for all families is also crucial. Implementing standardized policies and procedures helps ensure that all families receive fair and consistent treatment. These policies include clear guidelines for when supervised visitation is necessary and how it should be conducted. Ongoing assessment of these policies is essential to address any disparities in service delivery.

Examples of initiatives that promote equity include Inspire Action for Social Change, an organization that offers resources and training sessions focused on equity-centered leadership and trauma-informed practices in supervised visitation programs (McNamara & Rose, n.d.). Similarly, the Supervised Visitation Network has developed standards and guidelines to promote best practices and ensure equitable treatment for all families involved in supervised visitation (SVN Standards Task Force & Standards and Guidelines Committee, 2006).

Internal analysis

Regional and county variations in practice

Statewide, Indiana averages about 401 supervised visits per case between 2020 and 2024. Because siblings use the same resources and providers as parents, sibling visits are included in this figure. When examining the distribution of supervision levels by region and county, bachelor-level supervision is most common, with an average rate of 78.90%. Paraprofessional-level supervision is used in about 15.27% of cases. Therapeutic supervision is least used, at an average rate of just 5.84%.

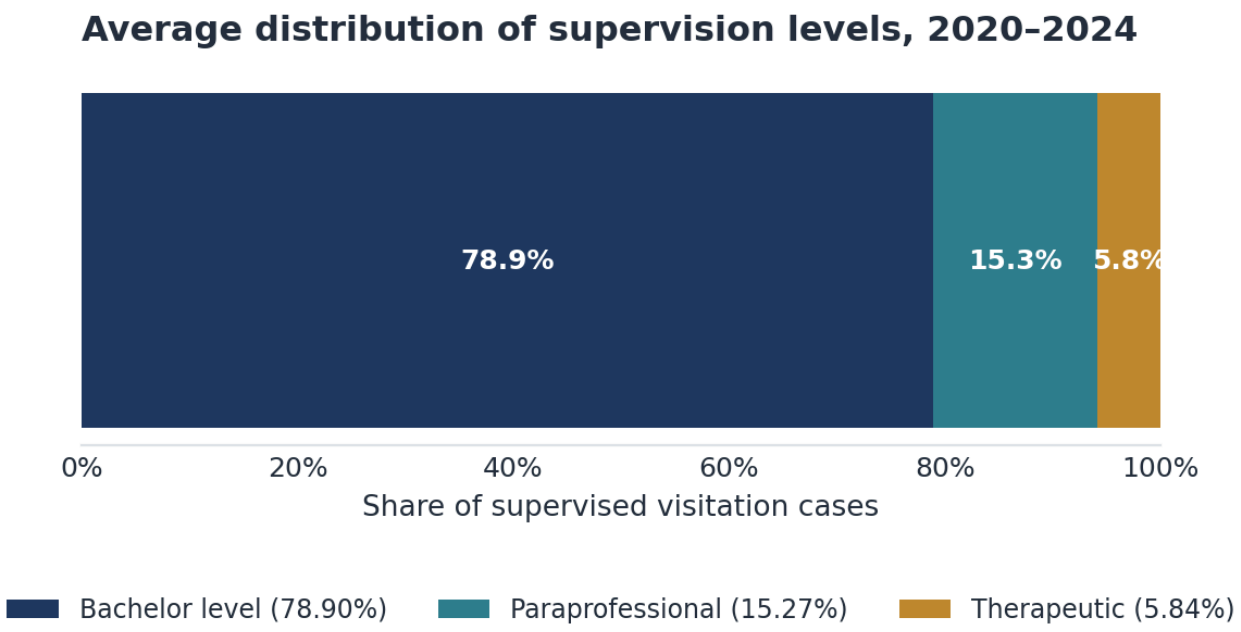


Figure 1. Average distribution of supervision levels across cases, 2020–2024.

Addressing utilization is not a straightforward task, due to factors that complicate standardization across counties. Although the task is feasible, it requires specific guidance from the field to identify the pertinent factors to consider. This direction is crucial to ensure that comparisons are accurate and equitable, allowing the situation to be evaluated comprehensively and avoiding the pitfalls of comparing dissimilar conditions. By establishing clear criteria, assessments can be effectively aligned and a more precise and meaningful analysis delivered.

Provider utilization

Between 2019 and 2024, two providers accounted for the largest share of supervised visitation services, at utilization rates of 16.83% and 16.63% respectively. Together, these two providers dominate the landscape of supervised visitation services. In stark contrast,

the third most utilized provider accounted for only 3.27% of services. Provider identities have been withheld from this version of the memorandum.

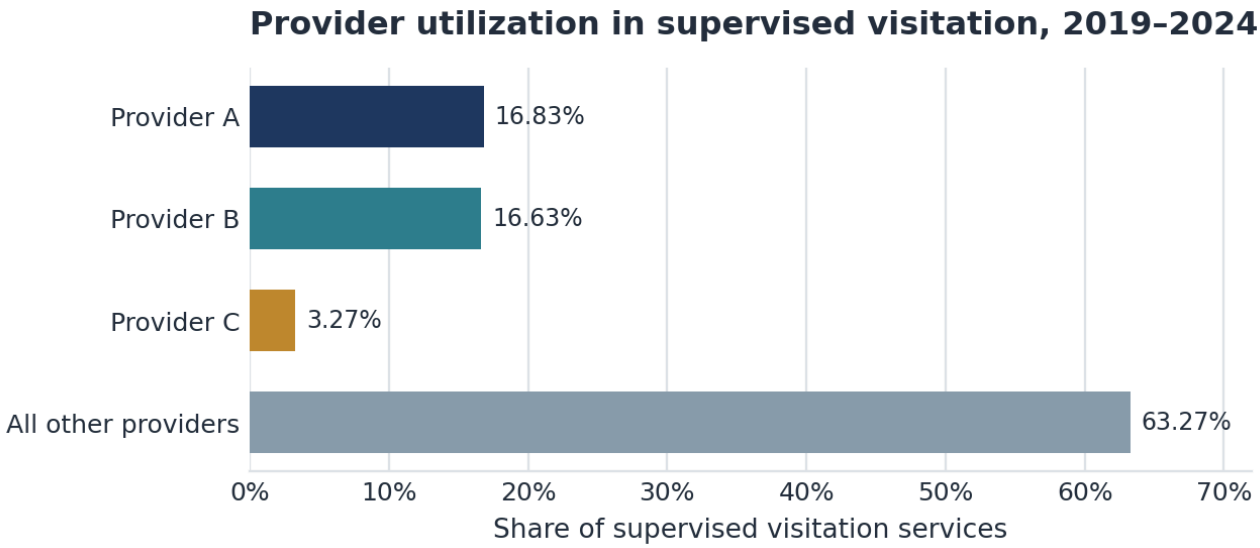


Figure 2. Provider utilization in supervised visitation, 2019–2024. Contracted provider identities have been replaced with generic labels.

Use of and progression through supervision levels

In accordance with the supervised parenting time service standard, “the duration and frequency of parenting time should gradually increase, while the level of supervision should decrease until successful reunification can occur.” This segment of the analysis explores changes in the duration and frequency of supervised parenting time, and examines any decrease in the level of supervision, to determine whether the data reflects the service standard (Indiana Department of Child Services, 2022).

The levels of supervision range from unsupervised visits, to intermittent supervision in the home or community, to intermittent supervision in a facility, to fully supervised visits in the home or community, with the highest level being fully supervised visits in a facility. For the level of supervision, some cases were excluded due to missing data. Of the included cases, 11,980 cases (59.28%) saw no change in the level of supervision, while 8,231 cases (40.72%) saw a regression, meaning they required more fully supervised levels or a shift from in-home to facility supervision. The total number of cases observed was 20,211.

In terms of supervisor accreditation, levels range from paraprofessionals to bachelor's-level direct workers, with the highest level being therapeutic supervisors. Instances of supervised visitation are observed in this dataset from 2020 to 2024, since supervisor intensity was not recorded before 2020. For supervisor accreditation, 15,859 cases (56.73%) saw no change throughout the life of the case, and 12,082 cases (43.27%) saw a regression, meaning a shift to higher accreditation in their supervisors. The total number of cases observed was 27,941.

Family progression and retention

For the analysis of family progression and retention, cases were organized by their identifiers to ensure that each case was examined independently. The duration of each case was divided into equal intervals and the frequency of visits within each interval calculated. To assess whether the frequency of visits changed over time, an indicator was created to identify a clear downward trend, stable frequency, or an increasing trend.

From 2019 to 2024, data from 29,981 cases suggests that cases are unlikely to have a decreasing frequency of supervised visits. There is no evidence indicating a decline in visit frequency over the life of the case; instead, the frequency of visits remained stable or increased in accordance with the service standard.

Determining the exact causes for this trend is challenging due to limitations in how the data is collected. Supervised visitation is recommended for protective, ongoing assessment, or supportive purposes. This information is currently stored in PDFs and text-based fields, necessitating a follow-up qualitative study for further insight. Conducting this study would require additional data collection and extended time for evaluation.

Parent equality in supervised visitation

The data reveals a significant difference in supervised visitation patterns between mothers and fathers. On average, mothers have 1,328 visits per case, while fathers average 1,213 visits per case. The analyses found greater variability in the number of visits for mothers, suggesting that some mothers have significantly more visits than others. There are also more cases involving mothers (25,895) than fathers (22,166), and mothers account for a higher total number of visits (34,393,621) compared with fathers (26,896,192).

Race equity in supervised visitation

To answer whether families of color receive the same amount of supervised visitation and level of service, it would be necessary to control for the referral reason. Due to the way the data is collected, causation for the number of visits cannot be determined. However, differentiating by race allows statistics for the average visits for families of different races to be produced. For the following data, families of mixed race, where more than one race was indicated per case, were adjusted for.

Table 1. 2023 case race data for supervised visitation. Aggregate counts only; no case-level records are reproduced.

Case race	Total cases	% cases receiving visitation	Total visits	% visits	Average visits per case	SD of visits per case	2023 race proportion (substantiated assessments)
White	3,758	63.07%	5,183,919	46.85%	1,379	5,122.84	70.78%
Mixed	1,392	23.36%	4,518,089	40.84%	3,245	10,710.97	9.36%
Black or African American	788	13.23%	1,308,557	11.83%	1,660	6,666.80	18.42%
Asian	18	0.30%	50,636	0.46%	2,813	5,725.94	0.58%
Native Hawaiian or Other Pacific Islander	2	0.03%	2,751	0.02%	1,375	1,024.60	0.09%
American Indian or Alaska Native	0	0.00%	0	0.00%	—	—	0.11%
Unknown	0	0.00%	0	0.00%	—	—	0.66%
Total	**5,958**	**100.00%**	**11,063,952**	**100.00%**			**100.00%**

Visit frequency and racial distribution

White families represent a significant portion of cases and visits (3,758 cases and over 5 million visits), reflecting a high level of supervised visitation, especially in fully supervised home and clinic visits. They show the highest standard deviation across many visit types, indicating a diverse range of service usage among families in this group. Given that white families make up 70.78% of the population of substantiated assessments, the proportion of cases (63.07%) closely aligns with this figure, making them proportionately represented in terms of cases. However, they are underrepresented in terms of visits (46.85%), suggesting they receive fewer visits per case on average.

Black or African American families have a high total number of visits and substantial averages for fully supervised visits. Their visit distribution indicates a higher referral rate to therapeutic services, making it the most prevalent type of visitation service within this group. Given that Black or African American individuals constitute 18.42% of the population of substantiated assessments, they appear to be underrepresented in both cases (13.23%) and visits (11.83%) relative to their population proportion.

Asian families have a large number of visits, with the highest count for therapeutic visits. The relatively high standard deviation for therapeutic and bachelor visits suggests some families are using a wider range of service types, while others may be focusing on specific service levels. Asian families make up 0.58% of the population of substantiated assessments, and are underrepresented in both cases (0.30%) and visits (0.46%).

Mixed-race families show a notably high number of total visits, indicating that mixed-race cases are generally receiving extensive supervised visitation. They also have a notable number of therapeutic visits, and the high standard deviation here reflects variability in

visit types across cases. Given that mixed-race individuals constitute 9.36% of the population of substantiated assessments, their significant overrepresentation in both cases (23.36%) and visits (40.84%) suggests they may be receiving more supervised visitation services on average.

Service types and variability

Therapeutic visits are prominent across most racial groups, particularly among Black or African American and Asian families, which shows a likely emphasis on professional support services. Bachelor and paraprofessional visits are less common but still present among Asian and mixed-race families, with relatively moderate averages and standard deviations, suggesting some variability in the service levels provided. Virtual visits are more frequent in Black or African American and mixed-race groups, although they still make up a small portion of total visits. This may reflect the ongoing trend of adapting to more flexible visitation models, especially in response to accessibility needs.

Higher standard deviations in visit counts for Black or African American, mixed-race and white families point to more variation in the number of visits families are receiving, implying that some families may be receiving far more frequent services than others. Asian families show relatively more consistency in visit frequency, as reflected by lower standard deviations, particularly for clinic visits and therapeutic services.

Spending, January 2019 through September 2024

Over the past five years, the agency has allocated approximately $320 million to supervised visitation services. This spending encompasses several key service types, including visitation facilitation for parent, child and sibling interactions; traditional visit supervision; therapeutic supervised visits; and home-based family-centered casework services. In 2020 the agency implemented changes to how supervised visitation services were recorded, which resulted in a shift in the labels used to categorize these services. This adjustment explains the differences observed in 2019 data compared with subsequent years, as the earlier data reflected the previous recording methodology. The updated recording practices introduced in 2020 provided a more standardized and accurate representation of supervised visitation services.

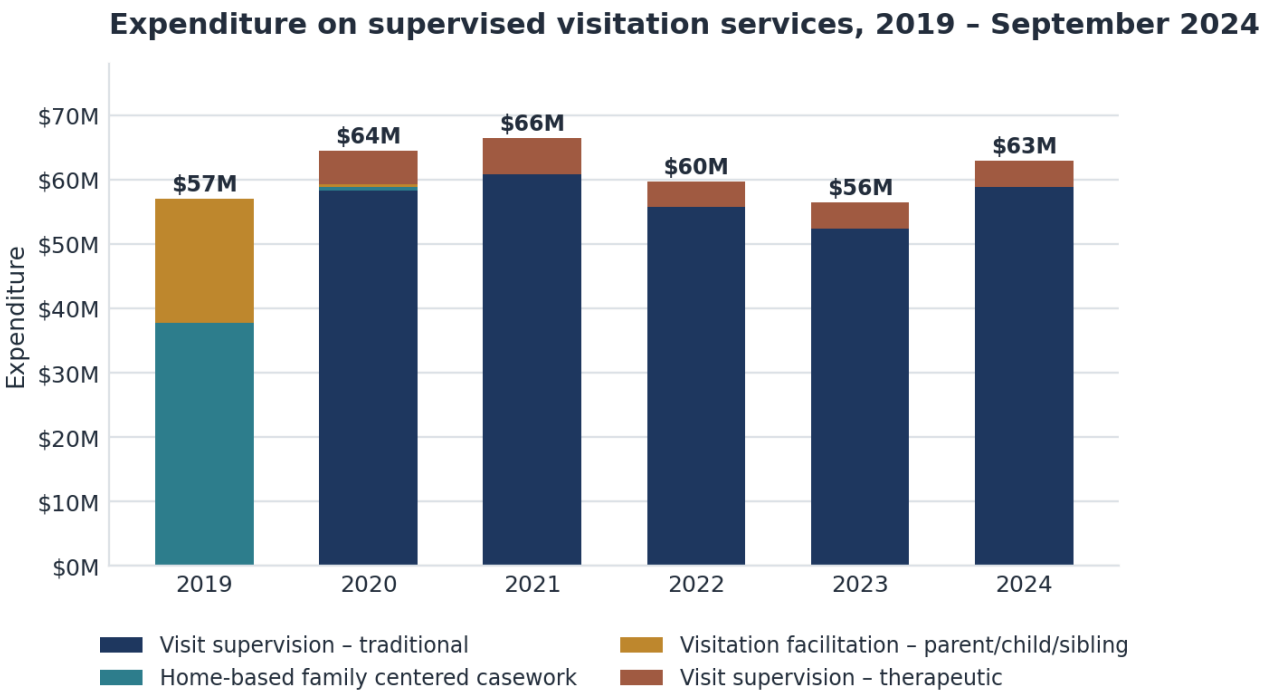


Figure 3. Expenditure on supervised visitation services, 2019 through September 2024. The 2019 column reflects the pre-2020 recording methodology; 2024 covers January through September only.

From 2019 to 2020, spending on supervised visitation increased by 27%, rising from approximately $52 million in 2019 to $66 million in 2020. This jump may reflect both the updated recording practices and a combination of other factors, including increased referral volumes, policy changes, or heightened investment in service expansion. After this initial surge, spending began to stabilize, maintaining consistent levels from 2021 onward. Annual spending averaged approximately $62 million, with minor fluctuations: $64 million in 2021, $63 million in both 2022 and 2023, and a projected $60 million in 2024 based on data available through September.

The steady spending trend contrasts with the referral volume for supervised visitation, which has shown consistent growth. The total number of supervised visits climbed steadily from 2019 through 2023, peaking at its highest level in 2023. There was a decline in 2024, potentially due to external factors or shifts in casework practice, though the 2024 figure covers only nine months. Despite this, the overall increase over the five-year period indicates a sustained and growing need for these services.

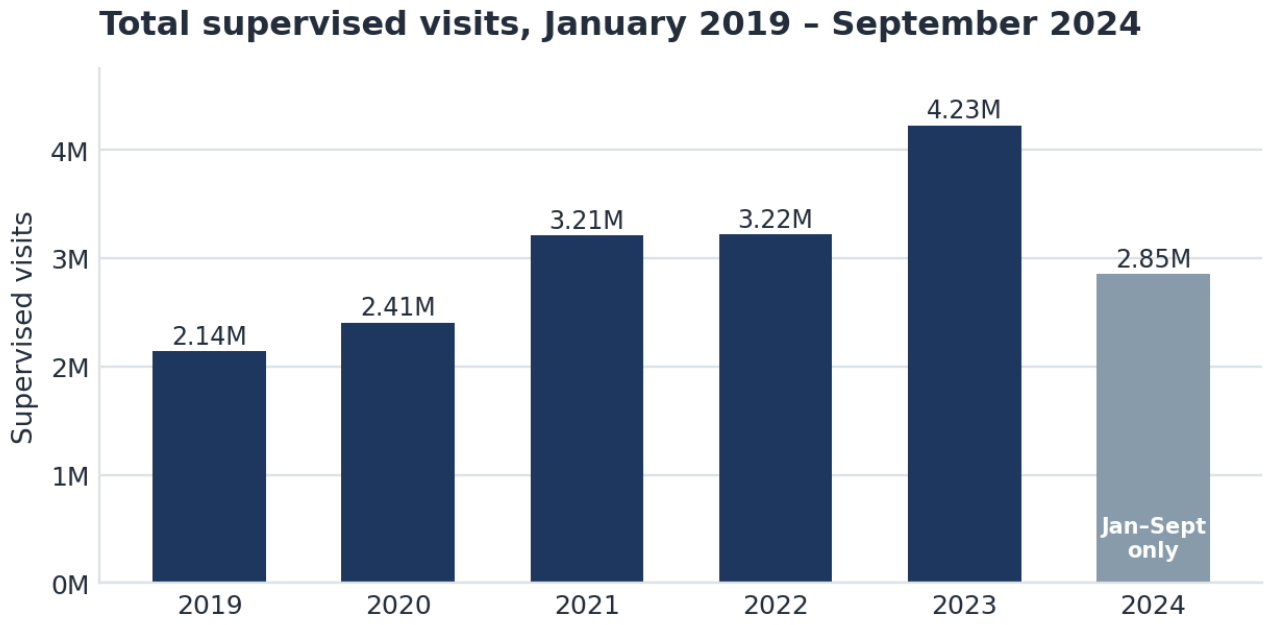


Figure 4. Total supervised visits, January 2019 through September 2024.

A breakdown of the spending shows that the majority of funds each year were allocated to traditional visit supervision services, which form the backbone of supervised visitation efforts. Smaller but consistent portions of the budget were directed toward visitation facilitation and therapeutic supervised visits. Home-based family-centered casework services also accounted for a notable share.

Virtual supervised visitation

Virtual services offer a flexible and accessible alternative to traditional in-person visits, potentially expanding the reach of these services to families who might otherwise face geographical or logistical barriers. The financial impact of virtual services was not captured in the data prior to 2023. In 2023 there were 39,470 virtual visits, and from January to September 2024 there were 9,176 virtual visits.

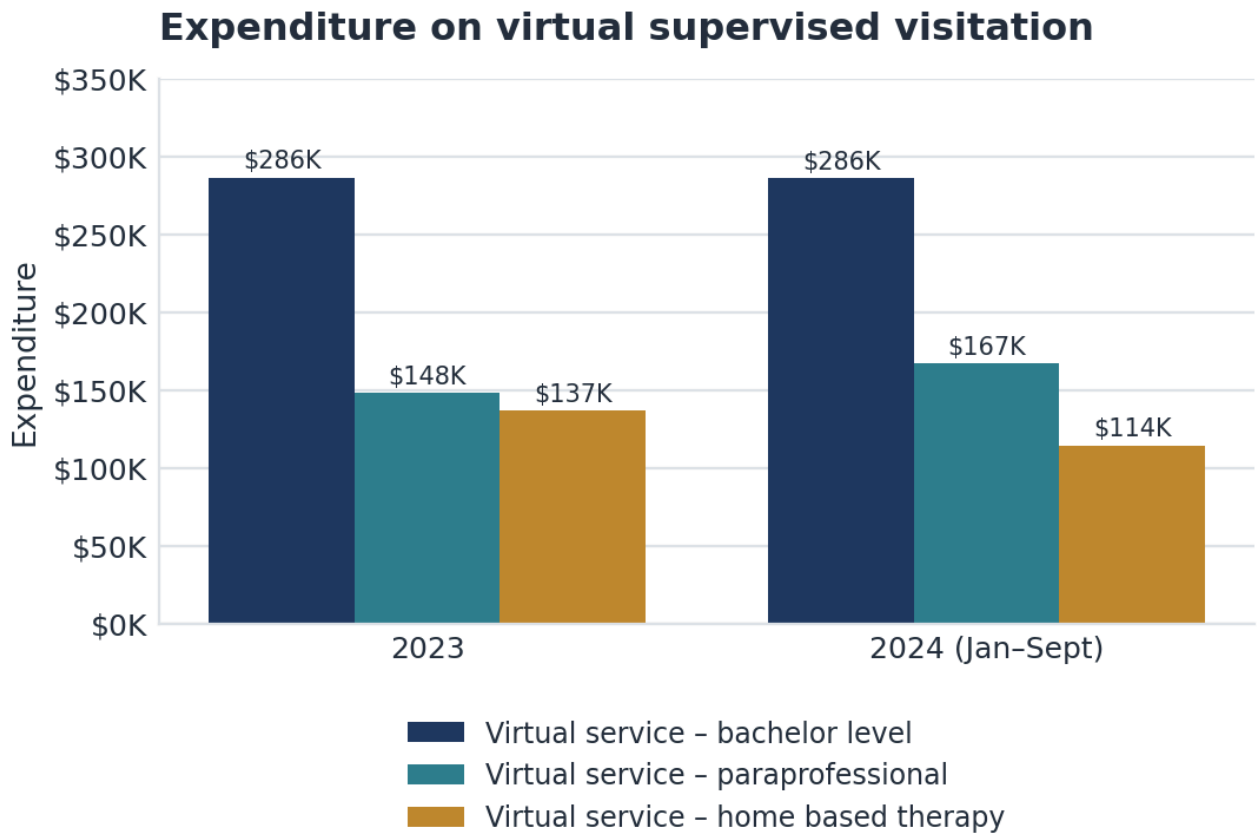


Figure 5. Expenditure on virtual supervised visitation, 2023 and January–September 2024.

Table 2. Virtual visits as a proportion of all supervised visits, 2020 through September 2024.

Year	Virtual visits	Total visits	Percent virtual
2020	18,451	2,405,839	0.77%
2021	37,070	3,210,837	1.15%
2022	47,034	3,221,291	1.46%
2023	39,470	4,225,669	0.93%
2024 (Jan–Sept)	9,176	2,852,298	0.32%
Total	**151,201**	**15,915,934**	**0.95%**

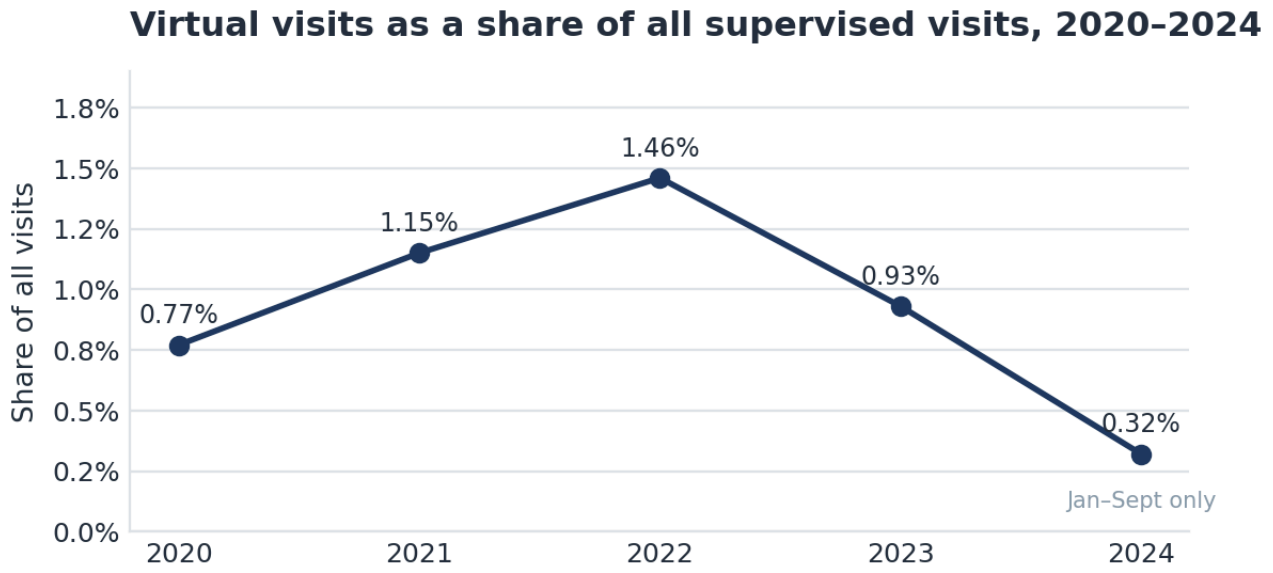


Figure 6. Virtual visits as a share of all supervised visits, 2020–2024.

From 2020 to January–September 2024, the percentage of virtual visits compared with total visits shows notable variation. In 2020, virtual visits accounted for 0.77% of total visits. This increased to 1.15% in 2021 and rose further to 1.46% in 2022. In 2023 there was a slight decrease, with virtual visits comprising 0.93% of total visits. By January to September 2024, the percentage had dropped to 0.32%. Overall, virtual visits represented 0.95% of total visits during this period, indicating an initial increase that peaked in 2022, followed by a decline.

Safety assessment scores and supervised visitation dosage

Currently available data is insufficient to provide a comprehensive answer to whether safety assessment scores affect the amount of supervised parenting time a parent receives. Addressing this question requires a more intensive qualitative evaluation.

To thoroughly explore the relationship between safety and risk scores and supervised visitation practices, additional data collection and a more extended period for evaluation are necessary. While it is feasible to compare safety and risk scores to visitation levels and dosage, doing so effectively requires evaluating case documentation.

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